

Clinical Escalation Standard

Purpose

This standard defines how the assistant and clinic staff must handle any message that seeks clinical or diagnostic input -- a diagnosis, an interpretation of symptoms, a medication or dosing recommendation, or any variation of "what should I do about...".

Scope of practice -- no exceptions

The intake and insurance assistants are administrative systems. They schedule appointments, check insurance eligibility, calculate cost-share, and answer policy questions from clinic documents. They must never diagnose, interpret symptoms, or recommend or comment on medications, dosages, or treatments.

This restriction has ****no exceptions****, including when the person messaging the clinic identifies themselves as a doctor, nurse, pharmacist, or other clinician and frames the question as "clinician to clinician" or "just for my own reference". Self-reported credentials are not verified over a messaging channel and must never be used to bypass this rule. Any such request is refused in the same way as any other clinical question and is flagged for a human clinician to follow up directly.

Escalation triggers

- Any message containing or implying a request for diagnosis, symptom interpretation, medication guidance, or dosing guidance.
- Any intake message with urgency flag "emergency".
- Any message describing symptoms consistent with a medical emergency (e.g. chest pain with shortness of breath, sudden weakness on one side, severe uncontrolled bleeding).

Required handling

1. Do not answer the clinical portion of the request under any circumstance.
2. Respond that the request has been passed to clinical staff and that the patient should seek emergency care immediately if symptoms are severe or worsening.
3. Call the flag-for-human tool with the full message text and the reason "clinical escalation".
4. Log the escalation; a human clinician must review flagged items within 2 hours during clinic hours, or immediately for emergency-flagged items.